



February 19, 2026

**VIA Electronic Submission to [www.regulations.gov](http://www.regulations.gov)**

Department of Health and Human Services  
Assistant Secretary for Technology Policy and Office of the National  
Coordinator for Health Information Technology  
Mary E. Switzer Building  
Mail Stop: 7033A, 330 C Street SW  
Washington, DC 20201

**Re: HHS Health Sector AI RFI - Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)**

Dear Secretary Kennedy,

On behalf of the Digital Pathology Association (“DPA”), please accept the following comments in response to the request for information (“RFI”) from the Assistant Secretary for Technology Policy (“ASTP”) and Office of the National Coordinator for Health Information Technology (“ONC”), entitled “Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care”.<sup>1</sup>

The Digital Pathology Association is a global community of over 4,500 pathologists, scientists, technologists, and industry leaders dedicated to the evolution of modern medicine. Founded in 2009, our mission is to facilitate awareness, education, and adoption of digital pathology and AI applications across healthcare and life sciences. By sharing best practices and demonstrating the efficiencies of new technologies, we empower our members to enhance workflows and, ultimately, improve patient care. Our impact is driven by a diverse international membership and guided by six core pillars: Collaboration, Education, Membership, Regulatory, Reimbursement, and Standards.<sup>2</sup>

Given our focus on revolutionizing patient care with digital pathology, we appreciate the opportunity to respond to a subset of the questions, providing our perspective on steps the Agency can take to drive innovation and the speed of adoption of digital pathology in patient care.

***1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?***

Innovators, providers, and patients face numerous different obstacles to the use of AI in clinical care including, but not limited to, the following:

- a. Investment and other costs of digital pathology are not systematically captured in reimbursement mechanisms developed and managed by CMS.*

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<sup>1</sup> See 90 Fed. Reg. 60108 ([link](#))

<sup>2</sup> <https://digitalpathologyassociation.org/>

In the development of digital pathology solutions, companies and laboratories incur meaningful research & development costs as well as other operational costs (related to image acquisition, computing and data storage, slide digitization, image analysis software, etc.). It is critical that these costs are captured and contemplated as CMS establishes inputs into and ultimate payment rates for these types of solutions under Medicare Part A and Part B. While CMS has nationally priced a number of different AI solutions – particularly under the Outpatient Prospective Payment System (OPPS) and the Physician Fee Schedule (PFS), the Agency has yet to take a formal position on its approach for rate-setting for these services. The absence of an established policy on its pricing methodology (i.e., how rates are established) translates to a lack of Agency transparency and predictability, a challenge that can serve as a deterrent or roadblock to innovation.

*b. Challenges in the timeline to and process for obtaining coding creates uncertainty.*

The three pillars of reimbursement are coding, coverage, and payment. The ability to obtain procedure codes that reflect the service furnished to patients is critical in order to obtain reimbursement. Historically, algorithmic analyses of previously performed diagnostic tests had pursued coding through the Proprietary Laboratory Analyses<sup>3</sup> pathway. This pathway was beneficial to laboratories and manufacturers for the following reasons:

- Limited time between code request and code effective date (significantly shorter than other pathways);
- Specificity of the code descriptor to appropriately describe the innovation, and;
- Proprietary nature of the codes.

Unfortunately, recent clarification by the AMA CPT Editorial Panel eliminated this coding pathway for many digital pathology solutions. As a result, there are fewer pathways available to obtain a procedure code and these pathways (Category III and Category I) have longer timelines and higher bars for success. By removing the option to pursue a PLA code, there is also greater uncertainty with respect to how these solutions will be viewed and thus priced by CMS.

**2. *What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.***

As the Agency is contemplating policy or programmatic changes that would facilitate the use and adoption of AI in clinical care, the DPA puts forth the following recommendations for consideration:

- Under the Outpatient Prospective Payment System, the New Technology APC payment pathway is a critically important option, particularly for AI-based solutions that do not qualify for transitional pass-through payment. To give stability and predictability to payment in the hospital outpatient

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<sup>3</sup> Proprietary Laboratory Analyses (PLA) codes were developed by the American Medical Association to identify specific and unique laboratory tests.

setting, CMS should set a minimum length of time of five years for all qualified services to remain in a New Tech APC.

- With OPPS, CMS has numerous different clinical APCs intended to represent services with similar clinical and resource requirements. Given key differences between AI-based and non-AI-based services, CMS should create a new clinical APC family for algorithmic services to ensure that solutions in the same APC are in fact similar or clinically comparable and with respect to resource requirements.
- For certain types of services, particularly ones that do not involve physician work, the cost for furnishing the care is comparable across the different sites of service (e.g., hospital outpatient, physician office). CMS should establish a process for quickly and consistently setting a payment rate under the Physician Fee Schedule that (a) reflects the costs of the innovation and (b) is consistent with the pricing approach taken by the Agency under OPPS.
- CMS/CMMI should establish a new demonstration model for AI solutions to help fund, facilitate, and highlight the clinical and potential cost benefits of digital pathology workflows. This pilot should apply across the different settings of care and should take into consideration the costs of these capital-intensive technologies that necessitate on-going investment and continuous refinement of the underlying models supporting the solution. The end goal of this model would be the establishment of coding, coverage, and payment pathways to ensure appropriate reimbursement and promote continued adoption of evidence-based AI technologies.
- For solutions recognized in guidance (e.g., National Comprehensive Cancer Network) or physician society clinical practice guidelines, the Medicare Administrative Contractors should expedite the establishment of positive, evidence-based local coverage determinations. The MACs have the flexibility to move quickly under the current regulations and should be encouraged to do so in these circumstances. These positive coverage policies would be beneficial not only to traditional Medicare beneficiaries but also the ever-growing Medicare Advantage patient population. Positive Medicare coverage policies will create a more favorable reimbursement environment, promoting appropriate, scientifically rigorous, and clinically meaningful AI innovation.

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The Digital Pathology Association appreciates the Agency's recognition of the need to accelerate the adoption and use of artificial intelligence as part of clinical care. We would welcome the opportunity to discuss these RFI comments with you in greater detail. Please feel free to contact Abbey Norris at 877.824.4085 or [ANorris@digitalpathologyassociation.org](mailto:ANorris@digitalpathologyassociation.org).

Sincerely yours,

/s/

Michael Rivers  
Chair, Reimbursement Taskforce  
Digital Pathology Association